

Dr James Carter
Family Dentist
Riverside Dental Practice
45 High Street
London

08/01/2024

Dear Dr Carter,

Re: Mr Adam Wilson

DOB: 15/03/1982

I am writing to update you on the recent ENT consultation of Mr Wilson, who will be referred back into your care following his diagnosis with odontogenic maxillary sinusitis. Your further dental management would be highly appreciated.

Today, Mr Wilson presented with a three-month history of left-sided facial pain increasing on bending forward, persistent unilateral nasal discharge and a foul nasal odour, which have not responded to repeated courses of oral antibiotics. These symptoms developed following a previous root canal treatment of his left upper molar. Please note, he denied having fever, epistaxis or visual disturbance.

On examination, there was tenderness over the left maxillary sinus, and purulent discharge was noted in the left middle meatus on nasal endoscopy. A CT scan of the paranasal sinuses demonstrated opacification of the left maxillary sinus with evidence of a periapical dental infection communicating with the sinus. Subsequently, Mr Wilson has been advised

to commence regular nasal saline irrigation and simple analgesia. However, definitive management requires treatment of the underlying dental source.

Mr Wilson's medical records show that he has type two diabetes mellitus and hypertension, for which he is on metformin and Norvasc. He drinks socially; however, he does not smoke.

I would be grateful if you could consider surgical intervention for Mr Wilson, if you find appropriate. If his symptoms persist despite dental treatment, combined ENT and dental surgical management may be required.

Should there be any queries, kindly do not hesitate to contact me.

Yours sincerely,

Dr Sarah Mitchell